

“Some additives have a formal Acceptable Daily Intake, but for many others, the real concern is repeated exposure through several foods consumed over the course of a day”

DR JASJEET WASIR, *Head, Endocrinology, Medanta - The Medicity, Gurugram*



“The problem is that one thinks if it’s soup, it must be healthy. But it is still out of a packet. Look closely at the label on the back, and you may find old concerns in new forms”

ISHI KHOSLA *Clinical Nutritionist, Delhi*



“The proposed FSSAI warning labels could make high levels of certain nutrients easier to identify, but label literacy at home remains important for making informed choices”

DR PEARLSY GRACE RAJAN, *Sr Consultant, Internal Medicine, Rela Hospital, Chennai*

make high levels of certain nutrients easier to identify, but label literacy at home remains important for making informed choices,” says Dr Pearlsy Grace Rajan, senior consultant - internal medicine, Rela Hospital, Chennai.

Take sugar, for instance. High-fructose corn syrup, glucose-fructose syrup and glucose syrup are all made by breaking down starch—commonly corn—into simple sugars. A 2025 systematic review and meta-analysis led by Prasetya Guntari found that fructose-glucose mixtures significantly increased fasting blood glucose and serum insulin in the trials analysed. Maltodextrin is another processed carbohydrate to be wary of. Made by partially breaking starch into much shorter glucose chains, it adds bulk, improves texture and carries flavours in seasoning powders, nutrition drinks, sauces and protein foods. In double-blind human trials published in the journal *Metabolites* in 2024, maltodextrin produced substantially greater post-meal glucose and insulin responses than a regular, slower-digesting carbohydrate in healthy people as well as participants with obesity or type 2 diabetes.

NOT SO SWEET

You may think you are getting rid of sugar, but its substitute could trigger a whole new set of problems. Sucralose, an intensely sweet derivative of sugar that contributes virtually no calories, has become ubiquitous in diet drinks, protein powders and ‘zero’ products. A small randomised, placebo-controlled trial published in *Clinical Nutrition ESPEN* in 2025 found that 30 days of sucralose consumption in healthy adults altered gut bacteria and was associated with higher post-meal glucose and insulin levels and a 20.3 per cent reduction in insulin sensitivity. The study did not suggest that everyday sucralose consumption causes diabetes—but it does challenge the assumption that a zero-calorie sweetener is necessarily metabolically inert.

“There is no single safe daily quota for the new ingredients appearing in ultra-processed food (UPF). Some additives have a formal Acceptable Daily Intake (ADI), but for many others, the real concern is repeated exposure through several foods consumed over the course of a day. That is why we are increasingly looking at how much ultra-processed food a person eats overall, rather than focusing on one additive in isolation,” says Dr Jasjeet Wasir, head of endocrinology at Medanta - The Medicity Hospital, Gurugram. This is particularly important because consumers cannot simply apply a universal “X grams a day” rule to every ingredient they encounter on a food label. An ADI is set for a specific substance and is based on an assessment of its safety over a lifetime. Staying below the ADI for an individual additive, therefore, does not make a diet dominated by UPFs healthy. As the World Health Organization notes, a healthy diet is based on adequacy, balance, moderation and diversity, with minimally processed and unprocessed foods as its foundation.

You may think you are getting rid of sugar, but its substitute like Sucralose, an intensely sweet sugar derivative, could lead to its own set of problems

That debate extends to aspartame and acesulfame-K as well. Khosla cautions that sugar substitutes may sound harmless, but that does not mean they should become an unrestricted, everyday part of the diet. Some observational studies have found an association between higher consumption of non-sugar sweeteners and a greater risk of type 2 diabetes, although researchers cannot say that the sweeteners themselves caused the increase. The evidence remains mixed, with short-term clinical trials generally failing to show the same effect.

Yet some studies suggest that the replacement sugars may affect more than just metabolic health. Erythritol, a sugar alcohol popular in low-carb, keto and sugar-free foods, attracted fresh attention after a 2024 human intervention study found that consuming 30 grams of erythritol acutely increased platelet reactivity and markers associated with blood clotting in healthy volunteers. The findings do not prove that erythritol causes heart attacks or strokes, but they add to earlier research linking higher circulating erythritol levels with cardiovascular risk and raise questions about its long-term safety. “Water, milk and minimally